

MyLoo

for patient advocates and gut-health creators - and what the research says about how to pitch it

"Photo in, AI classifies, clinic PDF out. Privacy-first. Earn from the value you already create."

You replied, thank you. This is the long-form companion to the email. It walks through what the research says about face-on-camera vs faceless content, then breaks down which path fits your creator archetype.

Contents: 1. App + business · 2. UGC vs faceless - what the research says · 3. Creator types table · 4. UGC strategy · 5. Faceless strategy · 6. Audience fit · 7. Cadence · 8. Conversion math · 9. Tracking summary · 10. Onboarding · 11. Dashboard · 12. Brand assets · 13. FAQ · 14. One last thing

1. App + business in 60 seconds

MyLoo is a digestion / stool tracker for iOS (Android launch on hold due to Google's Health Policy 2026). Photo of the sample, Claude Vision AI classifies on the Bristol Stool Scale 1-7, plus volume / texture / blood / mucus flags. Privacy-first: photos are NOT stored on our side, only the structured result lives in the timeline. The timeline carries symptom tags (bloating, cramps, urgency, pain level), you export a clean 30-day PDF for your GI appointment.

Why it sells

Word-doc tracking is the reality for 80% of IBS patients. MyLoo solves exactly that problem: at the GI appointment you no longer talk retrospectively for 4 weeks, you show a PDF with real data.

Free vs Premium

- Free: manual Bristol entry + symptom tags, timeline, manual PDF export.
- Premium (4.99 USD/m or 29.99 USD/y): AI photo classification, email-inbound parsing, extended clinic export.
- Free Lifetime Premium for onboarded affiliates, no posting obligation.

2. UGC vs faceless - what the research says

Face-on-camera content (UGC) converts higher than faceless content on average. For health-condition content, this gap is unusually large because trust matters more than scale.

Two findings worth knowing. Sokolova and Kefi (2020) showed that parasocial interaction - feeling like a creator is a friend - is a strong driver of purchase intent. For health-condition content this matters disproportionately: patients trust other patients more than aesthetic accounts. Lou and Yuan (2019) found that perceived creator credibility (trustworthiness, expertise) increases consumer trust; for IBS / IBD content, lived-experience visibility is the credibility signal. Industry benchmarks from Influencer Marketing Hub confirm that health-vertical UGC outperforms faceless by a wider margin than other verticals.

The caveat

Faceless still has a role for MyLoo. Educational explainers (the Bristol scale, symptom-tracking how-tos, low-FODMAP basics) work well as silent screen-rec or carousel format, and many patients with real conditions don't want to be on camera. Some of the most-watched IBS accounts on TikTok use anonymous-narrator format precisely because privacy is part of their value.

When each path works

UGC fits when YOU have the condition (IBS, IBD, post-surgery, food intolerance) and audience follows your lived experience. Faceless fits when you teach (Bristol scale, low-FODMAP), when anonymity is part of the brand, or when you create patient-education content as a healthcare professional in personal capacity.

Sources

- Sokolova, K., & Kefi, H. (2020). Instagram and YouTube bloggers promote it, why should I buy? How credibility and parasocial interaction influence purchase intentions. *Journal of Retailing and Consumer Services*, 53, 101742.
- Lou, C., & Yuan, S. (2019). Influencer marketing: How message value and credibility affect consumer trust of branded content on social media. *Journal of Interactive Advertising*, 19(1), 58-73.
- Influencer Marketing Hub. (2024). *The State of Influencer Marketing Benchmark Report 2024*.
- HypeAuditor. (2024). *State of Influencer Marketing Report*.

3. Creator types - which path fits you

Find your row. For MyLoo specifically, lived-experience creators convert dramatically higher than aesthetic accounts - health audiences see straight through performance. The conversion ranges below are realistic for the gut-health vertical.

Creator type	Primary path	Best hook angles	Realistic conversion
IBS / IBD day-in-the-life	UGC strong	Word-doc moment; "how I prep for my GI appointment"; the 30-day PDF reveal; "finally got a tool I trust"	3-5%, very high trust + share rate
Low-FODMAP / food-intolerance	UGC + Faceless mix	Food-symptom correlation timeline; "what I ate vs how I felt"; trigger-discovery story	2-4% in elimination-diet niches
Patient advocate / IBS-IBD educator	UGC	Educational explainer with personal data; "why the Bristol scale matters"; "what to bring to your GI"	2-4%, very high authority
Healthcare professional in personal capacity	UGC (with disclosure)	"What I wish my patients tracked"; clinical-grade tool walkthrough; "the PDF I want at appointments"	2-4% but must respect professional regulations
Anonymous gut-health TikTok	Faceless	Silent Bristol-scale education; symptom-tracking voiceless tutorial; privacy-first photo handling demo	1-2% per view but high posting cadence
Post-surgery / chronic-condition journal	UGC primary	Recovery timeline; "what helped me feel heard at appointments"; the value of structured tracking	2-3% in chronic-condition communities

4. UGC strategy (you on camera)

Who this fits

You with the condition. IBS patient sharing day-in-the-life. Post-surgery person tracking recovery. Patient advocate with the credibility your audience trusts. For health-condition content, audiences see straight through performance - your lived experience is the entire credibility model.

Hook examples that work

- "This replaced my Word doc" - talking head, you opening a real 4-week Word doc, then MyLoo timeline.
- "How I prep for my GI appointment now" - calm 60s walkthrough, ending with the PDF export.
- "Year one of IBS - what I wish I'd tracked from day one" - retrospective reel, MyLoo as the answer.
- "My GI doc actually engaged with the data this time" - emotional payoff reel, post-appointment debrief.
- "AI classified what I couldn't bring myself to look at" - privacy-first photo handling angle, vulnerability.
- "What changed in 30 days of tracking" - pattern-discovery story, you finding a trigger.
- "Why the Bristol scale matters and why nobody told you" - educational explainer with your data.
- "My low-FODMAP elimination week - what the timeline showed" - food correlation story.

Format templates

- Talking head 60s: direct to camera, your honest tone, app screen as B-roll.
- Day-in-the-life 90s: chronic-condition routine, MyLoo as a 5-second beat.
- Educational vlog 75s: teach one thing about gut health, demo the app to show the data.
- Emotional payoff reel: post-appointment debrief, the PDF moment.
- Retrospective: "year one with IBS - what I wish I knew", MyLoo answers the tracking-question.
- Comparison: Word doc vs MyLoo, your real-world before/after.

Do / Don't

- Do: only post if you actually have the condition or work in the space. Health audiences detect performance instantly.
- Do: name your condition or context specifically. "My IBS-D" reads true. "For people with gut issues" reads generic.
- Do: discuss privacy honestly. The photos aren't stored - say that, explain why it matters to you.

5. Faceless strategy (screen-rec, voiceover, B-roll)

Who this fits

Anonymous patient educator. Healthcare professional in personal capacity. Voiceover-only chronic-condition account. You're behind the camera, the education or the app demo carries the message. Many gut-health TikTok creators are intentionally anonymous because privacy is part of their value to audiences.

Hook examples that work

- Silent Bristol-scale infographic carousel - caption tells the story.
- 30s screen-rec of the AI classification, voiceover explains what the labels mean.
- "Privacy-first photo handling - what 'not stored' actually means" - explainer carousel.
- "What to bring to your first GI appointment" - tutorial carousel ending with the MyLoo PDF.
- "5 apps that actually help with IBS" - listicle, MyLoo at #1 or #2, no hard sell.
- Symptom-tracking time-lapse - anonymous hand showing app interaction.
- "The Bristol scale - three things most patients don't know" - silent educational reel.
- Comparison: Word-doc-vs-MyLoo split-screen, ambient music, captions only.

Format templates

- Educational carousel: 6 IG slides, calm clinical aesthetic, slide 6 = link.
- Screen-rec demo 20-30s: app does the work, captions explain.
- Voiceover narrative 45s: anonymous voice over screen-rec.
- Listicle: "X apps every IBS patient should know" with MyLoo at #1-2.
- Tutorial: step-by-step setup of the 30-day clinic export.
- Silent comparison: Word doc vs MyLoo, captions only.

Do / Don't

- Do: keep the clinical-calm tone. MyLoo's whole audience-fit is patients who hate wellness-influencer aesthetics.
- Do: caption every step. Sound-off viewing is the norm in health content.
- Do: cite credible sources when teaching (NHS, Mayo Clinic, Rome IV criteria for IBS). Generic claims sound suspect.
- Do: link in caption + bio. Health audiences read carefully and tap deliberately.
- Don't: use bright wellness-aesthetic colours. MyLoo's brand is deliberately clinical-calm.
- Don't: pretend to be a patient if you're not. Healthcare professionals in personal capacity work; fake-patient content destroys trust on contact.

6. Audience fit

Works well for

- IBS / IBD day-in-the-life creators (10k-200k, very high trust + share).
- Low-FODMAP food channels (food + symptom correlation angle).
- Patient-advocate IBS/IBD pages (medical authority).
- Post-surgery / chronic-condition journals.
- Anonymous gut-health TikTok accounts (privacy as brand).
- Healthcare professionals in personal capacity (nurses, dieticians, GI nurses).

Works poorly for

- Wellness-influencer aesthetic (clashes with MyLoo's clinical-calm brand).
- Diagnostic / 'AI tells you what's wrong' angle (MyLoo doesn't do this, never frame it that way).
- General lifestyle accounts without a real gut-health connection.

7. Posting cadence + best times

Gut-health audience: highest engagement Sunday evening ("prep-for-the-week") and Wednesday evening ("midweek"). IBS day-in-the-life creators see best results Tuesday-Thursday morning posts (audience checks first thing). Patient-advocate educational content performs all week, especially late evening 9-11pm. Frequency: 1 reel per week is plenty for affiliate conversion in this niche. Story sequences with link sticker after GI appointments drive strong conversion.

8. Conversion math (why this pays)

50% rev share, 24 months per attributed user. Premium 4.99 USD/m or 29.99 USD/y. IBS day-in-the-life micro (40k followers, 1 reel): 20,000 views x 2.5% CTR = 500 clicks, 40% install = 200 installs, 6% Premium = 12 subs, 12 months x 29.99 USD x 50% = ~180 USD per reel. Patient-advocate mid-tier (100k, 1 reel): 50,000 views x 2% CTR = 1,000 clicks, 35% install = 350 installs, 5% Premium = 17 subs, ~255 USD per reel. Anonymous gut-health macro (300k TT, 1 reel): 150,000 views x 1% CTR = 1,500 clicks, 30% install = 450 installs, 4% Premium = 18 subs, ~270 USD per reel. Conversion in this niche is unusually durable - chronic-condition users keep their subs longer than average, so the 24-month attribution window pays out fully more often than in lifestyle niches.

9. Tracking summary (full walkthrough in the Extended Brief)

Your link [myloo.org/i/\[yourhandle\]](https://myloo.org/i/[yourhandle]) works on every platform. iOS warm uses Universal Link (deterministic). iOS cold (~99% of creator traffic, given iOS-only) uses our DIY signed-token clipboard deferred-deeplink, live-verified, no Branch.io. RevenueCat fires on Premium purchase, 50% credited to your handle for 24 months from each user's first paid month. 30-day refund holdback before payout becomes claimable. Android attribution is built and tested but currently paused at the Google Health Policy step - affiliates can expect Android revenue once the corporate-account migration finishes. Full walkthrough in the Extended Brief PDF.

10. Onboarding (5 minutes)

- Click the personal link from Mail 2 (valid 7 days).
- Land on [myloo.org/affiliate/\[your-token\]](https://myloo.org/affiliate/[your-token]).
- Fill in: handle, Wise payout details, country (tax docs), billing address.
- Get your personal tracking link by mail + dashboard unlock.
- Free Lifetime Premium activated on your MyLoo account.

11. Dashboard (what you see)

- Live at getklar.org/dashboard (magic-link login).
- Clicks, installs (iOS warm / iOS cold; Android paused indicator), active Premium subs in your bucket.
- Claimable USD + paid USD + next payout date.
- Refund-holdback pipeline + per-app earnings funnel.

12. Brand assets (download)

Logo, app screenshots, Bristol-scale educational slides (medically reviewed), clinical-calm visual identity guide and ready-made captions: getklar.org/i/myloo/brand-assets (link goes live with your onboarding mail).

13. FAQ

I'm a healthcare professional, can I post about MyLoo?

Yes, in personal capacity with disclosure. Frame as 'a tool patients can use', not 'a tool that diagnoses'. Follow your local regulatory rules (UK GMC, German Berufsordnung, US state-level).

What about Android?

Android is built and tested but paused at Google's Health Policy step (corporate-account + D-U-N-S required). Your link still works for Android users - they hit a waitlist page. Once Android relaunches, your attribution applies retroactively to anyone who joined the waitlist.

Can I show real photos?

No, and you shouldn't want to. The privacy-first design means photos are never stored - that's the trust unlock. Use the app's structured-result view (timeline, Bristol number, tags) for any demo footage.

Disclosure obligation?

Yes, rev-share counts as paid recommendation. #ad / #sponsored / IG "Paid Partnership". Plus: if you're a healthcare professional, your local professional-conduct rules apply on top.

Flat fee?

Yes. Mid-tier \$150-500 per post, macros \$500+ (health vertical CPMs are above average). Rev share stays on top. Reply to Mail 2 to discuss.

What if I lose my niche credibility because of a sponsored post?

Don't post if MyLoo doesn't fit your actual lived experience or professional context. We'd rather miss the post than damage your credibility. That's a real answer.

14. One last thing

Honest read: MyLoo fits creators with real gut-health experience or professional context. If you don't have IBS, don't work in healthcare, and your audience isn't in a health niche, this is not a fit and we'd rather you not post. Patient audiences see through performance instantly. Reply to Mail 2 if you want to talk - yes, no, or 'maybe, let's see if my audience actually has this'. A 15-minute call goes a long way.

On posts with affiliate link: #ad / #sponsored / Paid Partnership (FTC, EU disclosure law).